

**ARBAMINCH UNIVERSITY
COLLEGE OF MEDICINE AND HEALTH SCIENCE
DEPARTMENT OF MIDWIFERY
COMPREHENSIVE QUALIFYING EXAM FOR BSC MIDWIFERY PROGRAM**

Name: _____

ID. No _____

INSTRUCTIONS:

- ✓ **THE QUESTION IS ORGANIZED IN NINE PARTS**
 - **PART I Obstetrics 50 Questions**
 - **Part II Gynecology 30 Questions**
 - **Part III Pediatrics & Neonatology 20 Questions**
 - **Part IV Reproductive Anatomy and Physiology 20 Questions**
 - **Part V Reproductive Health and FP 31 Questions**
 - **Part VI Epidemiology 11 Questions**
 - **Part VII Biostatistics 10 questions**
 - **Part VIII Health Service Management 7 questions**
 - **Part IX Health Assessment 10 questions**
- ✓ **The booklet has a total of 200 questions.**
- ✓ **All are Multiple Choice Questions. Each question carries equal mark (1 mark).**
- ✓ **Please read carefully each question and write the letter corresponding to your choice in the answer sheet attached here with.**
- ✓ **Write your Names in Each page & on the answer sheet provided.**

Time Allowed: 2:30 hours

GOOD LUCK!!!!

AMU, 2013

PART I: OBSTETERICS (50 QUESTIONS)

1. Aster is 25-years-old woman, gravida 2, para 1, presents for her first prenatal visit after testing positive on a home pregnancy test. She reports regular cycles every 35 days. She denies use of birth control pills, Depo-Provera, or other contraceptive in the last 7 months. The first day of her last menstrual period was April 1, 2007, and the last day was April 5, 2007. She says her periods always last 4 to 5 days. What is the best estimate of her due date?
A. January 1, 2008
B. January 8, 2008
C. January 12, 2008
D. January 15, 2008
2. A 27-year-old woman, gravida 4, para 3, at 30 weeks of gestation, presents to the clinic for a routine prenatal visit. Her pregnancy has been unremarkable thus far. Serosal fibroids are listed under her problem list. her fundus measures 37 cm from the symphysis pubis. In discussing possible complications of a fibroid uterus during pregnancy, you mention that she is at highest risk for:
A. Preterm premature rupture of membranes (PROM)
B. Pregnancy-induced hypertension (PIH)
C. Breech presentation
D. Placental abruption
3. One is **not** the sign that shows decent of the placenta.
A. The uterus becomes hard, round and moveable
B. Gush of blood
C. Uterus rises above the symphysis pubis
D. Lengthening of the cord
4. One of the following is not the characteristics of cervix immediately after delivery.
A. It is extremely soft, flabby and floppy
B. It may be bruised and edematous
C. It looks congested and admits two to three fingers with difficulty
D. Admit the tip of a finger
5. A woman 35 years old GII ParaI EFW of 3.6kg, presents with brow presentation. How do you manage her delivery?
A. Induction of labor
B. C/S
C. Observation to allow spontaneous rotation
D. Forceps assisted delivery
6. All are conditions for vacuum extraction EXCEPT;
A. Vertex presentation
B. Fully-dilated cervix
C. Fetal head at 0 stations
D. Descent 3/5
7. Descent Zero out of fifth means;
A. The head is floating
B. The head is fixed
C. Station is Zero
D. The whole head has entered the pelvic cavity

8. A 20-year-old woman (gravida 2, para 2) calls her Midwives 7 days postpartum because she is concerned that she is still bleeding from the vagina. It would be appropriate to tell this woman that it is normal for bloody lochia to last up to;
- A. 5 days
B. 8 days
C. 11 days
D. 14 days
9. Which of the following is not positive sign of pregnancy?
- A. Identification of fetal heart tones
B. Positive pregnancy test
C. Perception of fetal movement by the examiner
D. Identification of fetus by ultrasound
10. All of the following characteristics are applied to a pelvis favorable to vaginal delivery EXCEPT:
- A. Sacral promontory cannot be felt.
B. Obstetric conjugate is less than 10 cm.
C. Ischial spines are not prominent.
D. Sub-pubic arch accepts 2 fingers.
11. Which of the following terms best describes the pelvic type of small posterior sagittal diameter, convergent sidewalls, prominent ischial spines, and narrow pubic arch?
- A. Android.
B. Anthropoid.
C. Gynecoid.
D. Platypelloid.
12. The relation of the fetal parts to one another determines:
- A. Presentation of the fetus.
B. Lie of the fetus.
C. Attitude of the fetus.
D. Position of the fetus.
13. Which of the following is NOT a characteristic of normal labor:
- A. Progressive cervical dilation.
B. Increasing intensity of contractions.
C. Uterine relaxation between contractions.
D. Moderate bleeding.
14. According to the principles of Active Management, which of the following is NOT an indication that labor has started?
- A. Bloody show
B. Contractions - painful & regular
C. Effacement of the cervix
D. Rupture of membranes
15. As the baby's presenting part descends and puts pressure on the cervix, the mother's body responds by releasing increasing amounts of oxytocin, increasing the strength of contractions. What is this reflex called?
- A. Farquharson's reflex
B. Filigree reflex
C. Ferguson's reflex
D. Fusilim reflex
16. Which one of the following is not the function of placenta?
- A. Respiration
B. Digestion

- C. Excretion
 - D. Protection
17. During parturition, a positive-feedback mechanism occurs when ____ stimulates the secretion of _____, which results in uterine contractions.
- A. Progesterone, estrogen
 - B. Estrogen, oxytocin
 - C. Estrogen, progesterone
 - D. Oxytocin, estrogen
18. Which of the following is true?
- A. Fertilization is not usually takes place in the uterus.
 - B. Women can get pregnant only on the day of ovulation.
 - C. Males produce an average of one sperm a day.
 - D. Sperm survive for only one day after intercourse.
19. What will be the gestational age when a pregnant mother came to the clinic on 03/01/2005EC and her last menstrual period was 09/ 08 2004EC?
- A. 21⁺² wks.
 - B. 20⁺⁴ wks.
 - C. 149 days
 - D. 144 days
20. The MVA procedure is complete when;
- A. The wall of the uterus feels smooth
 - B. The vacuum in the syringe decreases
 - C. Red or pink foam and no more tissue is visible in the cannula
 - D. The uterus relaxes
21. Plotting on the partograph should begin;
- A. In the active phase of the first stage of labor
 - B. In the latent phase
 - C. When the cervix reaches full dilatation
 - D. When the woman is admitted to the labor ward
22. Cervical dilation plotted to the right of the alert line on the partograph indicates;
- A. Satisfactory progress of labor
 - B. Unsatisfactory progress of labor
 - C. The end of the latent phase
 - D. The end of the active phase
12. The vacuum cup should be placed;
- A. Above the flexion point, 1 cm anterior to the posterior fontanelle
 - B. Below the flexion point, 1 cm anterior to the posterior fontanelle
 - C. Over the flexion point, 1 cm anterior to the posterior fontanelle
 - D. Over the flexion point, 2 cm anterior to the posterior fontanelle
13. In a breech presentation, the fetal heart;
- A. Can usually be heard at a location higher than expected for a vertex presentation
 - B. Can usually be heard at a location lower than expected for a vertex presentation
 - C. Can usually be heard in the same location as for a vertex presentation
 - D. Is not able to be heard

14. In occiput posterior position;
- A. Vacuum extraction should not be performed
 - B. Vaginal birth cannot occur
 - C. The expulsive phase may be prolonged
 - D. The woman should not give birth in a squatting position
15. Diastolic blood pressure is 90 mm Hg or more before 20 weeks of gestation is symptomatic of;
- A. Mild pre-eclampsia
 - B. Chronic hypertension
 - C. Superimposed mild pre-eclampsia
 - D. Pregnancy-induced hypertension
26. An antihypertensive drug should be given for hypertension in severe pre-eclampsia or eclampsia if diastolic blood pressure is;
- A. between 100 and 110 mm Hg
 - B. 110 mm Hg or more
 - C. 115 mm Hg or more
 - D. 120 mm Hg or more
28. Tears of the cervix, vagina or perineum should be suspected when there is immediate postpartum hemorrhage and;
- A. A complete placenta and a contracted uterus
 - B. An incomplete placenta and a contracted uterus
 - C. A complete placenta and an atonic uterus
 - D. An incomplete placenta and an atonic uterus
29. If a retained placenta is undelivered after 30 minutes of oxytocin administration and controlled cord traction and the uterus is contracted;
- A. More aggressive controlled cord traction should be attempted
 - B. Controlled cord traction and fundal pressure should be attempted
 - C. Manual removal should be attempted
 - D. Ergometrine should be given
30. After the cup is properly positioned over the fetal head, effective traction usually requires pressure:-
- A. Less than 0.6 kg per cm²
 - B. More than 0.8 kg per cm²
 - C. 0.6 kg per cm² - 0.8 kg per cm²
 - D. Less than 450 mm Hg
31. General management of the woman who develops a fever after childbirth includes
- A. Bed rest
 - B. Adequate hydration by mouth or iv
 - C. Use of a fan or sponging with tepid water
 - D. All of the above
32. If there is continued heavy bleeding after manual removal of the placenta
- A. Ergometrine 0.2 mg should be given by mouth
 - B. Ergometrine 0.2 mg IM should be given IM
 - C. Oxytocin 10 units should be given as an IV bolus

- D. Prostaglandin 2.5 mg IM should be given IM
33. The most common type of uterine incision which usually affords adequate access to effect delivery and provides for the strongest postoperative repair is:-
 A. Low transverse incision
 B. Low vertical incision
 C. **High vertical incision**
 D. Classic uterine incision
34. Which one of the following is the most common complication of obstetric forceps?
 A. Uterine rupture
 B. **Trauma/ soft tissue damage**
 C. Intracranial hemorrhage
 D. Skull fracture
35. In which degrees of the placenta previa, vaginal delivery is attempted before making decision to undergo cesarean delivery?
 A. Type 2 when placenta is posterior
 B. Type 3 placenta previa
 C. Type 4 placenta previa
 D. **Type 2 when placenta is anterior**
36. Which one of the following is not an indication of the destructive vaginal operation?
 A. Breech delivery
 B. **Ruptured or impending rupture uterus.**
 C. Transverse lie
 D. Cephalo-pelvic disproportion
37. What is the **most common** malposition in obstetrics?
 A. **Occipito posterior position**
 B. Occipito anterior position
 C. Sacro posterior position
 D. Occipito transverse position
38. The fertilized ova division in monozygotic twins occurs **before 72hours**, what features you expect for this identical twins?
 A. **One placenta with monoamniotic & monochorionic**
 B. One placenta with diamniotic & monochorionic
 C. One placenta with diamniotic & dichorionic
 D. One placenta with monoamniotic & dichorionic
39. All of the following issues are **not always** true about *cord prolapse* **except**;
 A. The umbilical cord lies in front of the presenting part
 B. **Bag of water is ruptured**
 C. Auscultate abnormal fetal heart rate
 D. The risk are greater in breech than cephalic presentation
40. W/ro Belaynesh came to in your clinic when she had not expelled her placenta, since the fetus was delivered before 3 hours. When you were checking her vital sign you had got Hypotension & Tachycardia suddenly she develops shock, WHAT TYPE OF SHOCK IS IT?
 A. Anaphylactic shock
 B. **Hypovolemic shock**
 C. Cardiogenic shock
 D. Neurogenic shock

41. The woman who was in labour came to in Arba Minch Hospital in labour ward during your practical session. This woman admitted by you. When you were performed **Per-Vaginal examination** you were appreciated cephalic presentation with the largest diameter. WHAT TYPE OF PRESENTATION SHE HAVE?
- Breech presentation
 - Face presentation
 - Brow presentation**
 - Shoulder presentation
42. Marta who is 20 years old woman was delivered her 1st birth in your health institution without any major complication. After 2 hours you want to discharge the woman & when you are going to told for her the woman is very sad, anxiety, irritation, restlessness, confused, and crying without any reason. WHAT IS YOUR IMMEDIATE DIAGNOSIS?
- Postpartum blues**
 - Postpartum psychosis
 - Postpartum depression.
 - postpartum hemorrhage
43. W/ro Almaz had 2 Hospital deliveries one SVD (spontaneous vaginal delivery) and one C/S delivery (because of obstructed labour since the fetus was hydrocephalus) previously. Now she is in your clinic to give birth for the 3rd time, but the labour exceeds 24 hours after the onset of true labour. WHAT IS YOUR EXPECTATION FOR THE CAUSE OF PROLONGED LABOUR ACCORDING TO HER PREVIOUS HISTORY?
- | | |
|---------------------|-------------------------|
| A. Passenger | C. Powers |
| B. Passages | D. Psychological causes |
44. A multy gravida woman came to in your clinic with 2nd stage of labour, & the fetus is delivered after 2 hours of 2nd stage of labour with a little bite distress. When you are trying to deliver the placenta excessive bleeding is occurred & your assistance checks her vital sign of hypotension & tachycardia results are recorded, suddenly maternal respiratory distress and convulsions are happened. WHAT WILL BE THE PRIMARY POSSIBLE DIGNOSIS OF THIS COMPLICATION?
- Puerperal Sepsis
 - Thrombophlebitis
 - Amniotic fluid embolism**
 - Puerperal Psychosis
45. Which features is not always true in monozygotic twins?
- Same sex
 - Same gene & blood group
 - Same physical features(eye & hair color, ear shapes)
 - Same body size**

46. Which type of episiotomy is best recommended for midwives after indications are visible?
- A. Median episiotomy
 - B. Medio-lateral episiotomy**
 - C. J incision
 - D. Inverted T- incision
47. Which one of the following types of skin closure after episiotomy is recommended currently due to cosmetic purpose?
- A. Sub-cuticular suturing**
 - B. Continuous suturing
 - C. Interrupted suturing
 - D. Intermittent suturing
48. Which one of the following types of destructive vaginal deliveries is considered in cases of shoulder dystocia after other maneuvers have been unsuccessful?
- A. Decapitation
 - B. Evisceration
 - C. Craniocentesis
 - D. Cleidotomy**
49. Which one of the following is not the characteristic of Traumatic Postpartum Hemorrhage?
- A. Firmly contracted Uterus
 - B. Bright red bleeding Starts immediately
 - C. Bleeding is from the placental site**
 - D. Bleeding is not treated with oxytocic drugs
50. How much ml bleeding is considered as PPH in C/S delivery?
- A. >500ml
 - B. >1000ml**
 - C. 500-1000ml
 - D. 300-500ml

PART II GYNECOLOGY (30QUESTIONS)

51. Which one of the following is not the risk factor for cervical cancer?
- A. Infection with HPV subtypes 16 and 18
 - B. long-term (> 5 years) use of oral contraceptives
 - C. use of Intrauterine devices (IUDs)**
 - D. Early onset of sexual activity and multiple sexual partners.
52. Which one of the following is the most common clinical manifestation of the cervical cancer?
- A. Bowel or urinary symptoms, such as pressure-related complaints, hematuria or hematochezia
 - B. Vaginal passage of urine or stool
 - C. Irregular or heavy vaginal bleeding usually post coital and vaginal discharge**

- D. Pelvic or lower back pain, which may radiate along the posterior side of the lower extremities.
53. The most common histologic types of cervical cancer is:-
- A. Adenocarcinoma
 - B. Squamous cell**
 - C. Adenosquamous carcinoma
 - D. Undifferentiated carcinoma
54. Which one of the following is not the initial treatment option for patients with Cervical intraepithelial neoplasia (CIN)?
- A. Ablation therapy and Excision therapy
 - B. Expectant management
 - C. Topical treatment with cidofovir
 - D. Hysterectomy**
55. Which one of the following is a component of cervical cancer control?
- A. Primary prevention of HPV infection and cofactors known to increase the risk of cervical cancer
 - B. Early detection by organized screening programs, targeting the appropriate age group and with effective links between all levels of care
 - C. Treatment of precancer, using relatively simple procedures, to prevent the development of cancer and treatment of invasive cancer, using surgery, radiotherapy and chemotherapy
 - D. All**
56. Which one of the following does not increase the risk of GTD?
- A. Extremes of maternal age (over age 35 years)
 - B. Maternal blood type AB, A or B
 - C. Extremes of paternal age**
 - D. Deficiency in vitamin A
57. The most common Clinical manifestations of GTD regarding its frequency is:
- A. Enlarged uterus
 - B. Vaginal bleeding**
 - C. Hyperemesis gravidarum
 - D. Preeclampsia before 20 weeks of gestation
58. Which one of the following is not true about endometrial cancer?
- A. Endometrial cancer is the most common gynecologic cancer and the fourth most common cancer in women.
 - B. Most endometrial cancers are thought to arise as a result of increased exposure to unopposed estrogen.
 - C. Endometrial cancer is predominantly a disease of postmenopausal women and the average age for developing endometrial cancer is about 60 years.
 - D. Endometrial cancer spreads most commonly by hematogenous dissemination.**

59. The origin of the majority of ovarian cancers are:-

- A. Germ cell
- B. sex-cord/stromal
- C. Epithelial
- D. Metastatic tumors to the ovary

60. Which one of the following is not true about breast?

- A. Women aged greater than 40 years should have screening mammography every 1–2 years and Women aged greater than 50 years should have annual screening mammography.
- B. All women older than age 20 years should be advised to do breast self-examination monthly to detect palpable breast cancer.
- C. Most breast cancers occur in the upper outer quadrant and sub areola region because that is where most of the tissue is located.
- D. Almost all neoplastic breast pathology arises far apart from terminal ductal lobular units (TDLU).

61. Which one of the following approaches that women can take to decrease their risk of breast cancer?

- A. Minimizing use of postmenopausal hormone therapy.
- B. Childbearing at a younger age and breastfeeding for at least six months.
- C. Avoidance of adult weight gain, limiting alcohol consumption, avoidance of smoking and regular physical activity.
- D. All

62. Which one of the following are the signs and symptoms of cervical incompetence?

- A. One or more mid-trimester spontaneous abortions, often preceded by painless rupture of the membranes.
- B. An unusually patulous cervical os easily admitting the index finger and during the pregnancy the membranes may be seen to protrude from the cervical os.
- C. A history of previous cervical surgery or a previous rapid delivery with cervical trauma.
- D. All

63. Which of the following is not true about physiologic menses?

- A. The amount of blood flow is 10 - 80 ml during reproductive age life.
- B. The frequency of the menses to occur is cyclic that ranges from 21- 35 days between each interval.
- C. The color of blood flow is bright red and clotted due to the presence of plasmin.
- D. The menses flows on average for 4 – 7 days

64. The treatment of choice for anovulatory DUB is:-

- A. NSAIDs
- B. Danazol
- C. Oral contraceptive
- D. Hysterectomy

65. The most common cause of primary dysmenorrhea is

- A. Endometriosis.
- B. PID
- C. Leiomyoma,
- D. Psychological stress
- E. **Prostaglandins**

66. Which of the following statements is CORRECT in regard to ectopic pregnancy?
- A. The incidence of heterotopic pregnancy has declined in recent years due to the increasing use of assisted reproductive technologies.
 - B. Primary abdominal pregnancies occur more commonly than secondary abdominal pregnancies.
 - C. The prior history of ectopic pregnancy does not increase the risk of subsequent ectopic pregnancy.
 - D. **Failure to visualize a uterine pregnancy by TVS with a serum hcG above discriminatory level is 100 percent accurate in excluding a live uterine pregnancy.**
67. Which of the following is the preferred primary treatment in patients with metastasis and a high risk prognostic score of GTD?
- A. Triple therapy with MTX, Act-D and cyclophosphamide.
 - B. **EMA-CO**
 - C. A single chemotherapy drug (methotrexate or dactinomycin).
 - D. B and C
68. Semira is 19 years old nulliparous comes to the health institution with the complain of slight vaginal bleeding, vomiting, visual disturbance, severe head ache and chest pain. She has not been seen her menstrual cycle since 4 months. On physical examination of the abdomen the size of her uterus is at the umbilicus, on hearing to fetal heart sounds are absent and there is no other pertinent finding on the general physical examination. Her vital sign is normal except here BP which it is 145/95mmHg and on laboratory investigation the urine she is positive for both hcG and albumin. What would be the possible diagnosis for semira's problem?
- A. Menstruation disorder(amenorrhea)
 - B. Missed abortion
 - C. Secondary abdominal pregnancy
 - D. **Complete hydatidiform mole**
69. A 23-year old primigravida presents to the emergency room at 15 weeks' gestation having experienced a gush of fluid. On examination amniotic fluid is found in the vaginal vault and the cervix is 3cm dilated with fetal parts seen. What type of abortion is this?
- A. Threatened abortion
 - B. Incomplete abortion
 - C. **Inevitable abortion**

70. Which one of the following management plans most appropriate in the treatment of unstable ectopic pregnancy?

71. Which one of the following is FALSE about gestational trophoblastic disease?

72. What is the most common karyotypic abnormality seen in spontaneous abortion?

73. Which of the following drug therapy is used to induce ovulation in the infertile women Failure to ovulate?

- ### D. A11

74. Which of the following is NOT CORRECT about hormonal changes in menopause?

75. Hormonal replacement therapy by estrogen is not effective for the following health problem arises after menopause?

76. All of the following should be part of routine evaluation and care of all women with possible miscarriage, except

- 11

- B. Speculum examination and Urine or serum pregnancy test
- C. Evaluation for adnexal masses and tenderness
- D. RhoGAM injection

77. A 58 year old woman complains of urinary leakage after exertion. She loses urine after coughing, sneezing, exercising, and taking. She underwent menopause five years ago. She is not on ERT. On physical exam urethral detachment with a +ve Q-tip test has been seen. What are Potential diagnoses?

- A) Stress incontinence
- B) Urge incontinence
- C) Urinary incontinence
- D) Overflow incontinence

78. One of the following is/are true regarding the treatment of uterine fibroids:

- A) Birth control pills are used to reduce the size of fibroids.
- B) Depo-lupron – is recommended for 7 months.
- C) Radical hysterectomy- when individual fibroids are cut off.
- D) Supra cervical hysterectomy-the cervix is left in place perhaps the uterus has been removed totally.

79. If a woman with a nine week pregnancy has uterine contractions that become increasingly painful and had excessive vaginal bleeding. The cervical dilation is open and effaced, but no tissue has passed. The most likely diagnosis is?

- A. Incomplete abortion
- B. Missed abortion
- C. Threatened abortion
- D. Inevitable abortion

80. The underlying pathophysiology of pelvic organ prolapse is which of the following?

- A. Inherent connective tissue weakness
- B. Childbirth injury
- C. Aging
- D. All of the above

PART. III PEDATRICS & NEONATOLOGY (30 QUESTIONS)

81. Immediate resuscitation using a bag and mask should be implemented if the baby's respiratory rate is?

- A. between 30 and 60 breaths per minute
- B. less than 40 or greater than 30 breaths per minute
- C. between 20 and 30 breaths per minute
- D. less than 20 breaths per minute

82. One of the following is wrong about hypoglycemia in new born.

- A. Affects brain development
- B. Affects oxygen consumption in the brain
- C. It could be the resulted of pancreas adenoma
- D. It does not potentiate break down of amino acids

83. Red umbilicus in neonate indicates based on IMNCI Classification.

- A. Very severe diseases
- B. Local infection unlikely
- C. Very severe diseases unlikely
- D. Local bacterial infection

84. Which of the following neonates are commonly affected by intraventricular hemorrhage?
 A. Post term
 B. Premature
 C. Term
 D. Un known
85. Where does suctioning should start during resuscitation of the new born?
 A. Mouth
 B. Nose
 C. Either of
 D. Unknown
86. The advantages of early initiation of breast feeding to the newborn?
 A. To avoids infection
 B. To increase the immunity
 C. To prevent from hypoglycemia
 D. All
87. Which one is **incorrect** procedure to give immediate newborn care?
 A. application of butter /dung on the umbilicus
 B. dressing the cord
 C. bathing before 24hrs
 D. all
88. The recommended medication given as a prophylaxis to prevent STI from mother to newborn?
 A. TTC
 B. Vit k
 C. BCG
 D. A and B
89. Which one is not the management of jaundice in the newborn?
 A. delay feeding
 B. Phototherapy
 C. Adequate hydration
 D. Exchange transfusion
90. Which types of viral infection during pregnancy is common to causes newborn cataract, glaucoma, heart defects and deafness?
 A. Cytomegalovirus
 B. Toxoplasmosis
 C. Rubella
 D. Herpes simplex
91. W/o Tigist came to in your clinic with her male newborn to vaccinate him OPV0 since she was gave birth in home before 5 days but when you diagnose him he looks like yellowish discoloration of the skin, sclerae, mucous membranes and nails. What will be the primary finding from your diagnosis?
 A. Hyperthermia
 B. Hyperglycemia
 C. Hyperbilirubinemia
 D. Hypothermia
92. Which one is the most common site used to monitor the newborn's temperature?
 A. Tympanic
 B. Oral
 C. Axillary
 D. Rectal
93. APGARs score includes all the followings EXCEPT:
 A. Skin color.
 B. Blood pH.
 C. Heart rate.
 D. Muscle tone
94. The heart rate of a normal fetus at term:
 A. 80-100 bpm.
 B. 160-180 bpm.
 C. 100-120 bpm.
 D. 120-160 bpm.
95. Which of the following is drug of choice in the management of neonatal tetany?

- A. Potassium chloride
B. Sodium chloride
C. Ferrous sulfate
D. Calcium gluconate
96. The preterm newborn encounters the problem of severe heat loss because of the following reasons, **except**:
- A. The baby's large body surface area relative to body mass
B. The tiny baby's small size presents a much bigger heat sink to store thermal reserve
C. Minimal shivering during exposure to cold
D. Poor reserve of brown fat
97. Which of the following MUAC measurement shows malnutrition in child age above 6 months?
- A. <11cm
B. <12cm
C. <13cm
D. <15cm
98. ABO blood group incompatibility occur and result in autoimmune hemolytic anemia in children if:
- A. The mother's blood type is O, A or B
B. The fetal blood type is same with mother's blood group
C. The fetal blood type is O
D. The mother's blood type is AB
99. The single most common site of bleeding due to hemophilia is:
- A. Knee
B. Brain
C. Respiratory organ
D. Shoulder
100. Yared is 12 years old and his teacher ordered him to read the paragraph he is teaching. Yared started to read but suddenly interrupted reading without aura and stares blankly for about 30 seconds but he did not lose his body tone. His teacher was talking to Yared to continue reading but he was not aware what is going on, after about 30 seconds Yared started to read without postictal state. Which type of seizure did Yared have?
- A. Grand Mal
B. Absence
C. Myoclonic
D. Clonic
101. Which one of the following clinical presentation of type one DM is the most common?
- A. Polyuria
B. Polydipsia
C. Rapid weight loss
D. Polyphagia
102. Which one of the following anatomical sites is appropriate for children to assess pallor during physical examination if you suspect anemia?
- A. Mucus membrane
B. Nail beds
C. Conjunctiva
D. All are correct
103. What is the most sensitive vaccine to heat?
- A. BCG
B. OPV
C. PCV10
D. Measles
104. Amanuel is 9 year old Uncircumcised and complaining abdominal or flank pain, urgency, dysuria, frequency, incontinence, malodorous urine and constipation. He has no history of Catheterization, genital manipulation and receiving broad-spectrum antibiotics .On physical examination costovertebral tenderness is noted and vital signs are in normal range for his

age. In the urine examination by microscopy hematuria and > 5 WBC/HPF is noted. What will be the possible diagnosis for the Amanuel's problem?

- A. Acute pyelonephritis
 - B. Asymptomatic Bacteruria
 - C. Acute cystitis
 - D. Appendicitis
105. Which of the following is the most common cause of CHF in the first week of birth life and premature infants?
- A. Asphyxia
 - B. Acute hypertension
 - C. Fluid overload
 - D. Severe anemia
106. State of being protected against disease through the presence of antibodies either by previous infection or by vaccine.
- A. Immunity
 - B. Immunization
 - C. Vaccine
 - D. Vaccination
107. What is the expected problem when their physical diagnosis result is between 60 and 80% weight for age with edema?
- A. Kwashiorkor
 - B. Undernutrition
 - C. Marasmic-Kwashiorkor
 - D. Marasmus
108. The newborn is usually comfortable with one of the following senses?
- A. Touch
 - B. Smell
 - C. Hearing
 - D. Taste
109. Red umbilicus in young infant indicates based on IMNCI Classification.
- A. Very severe diseases
 - B. Local infection unlikely
 - C. Very severe diseases unlikely
 - D. Local bacterial infection

110. Which of the following infants are commonly affected by intraventricular hemorrhage?
- A. Post term
 - B. Premature**
 - C. Term
 - D. Un known

PART. IV. REPRODUCTIVE ANATOMY & PHYSIOLOGY (20 Questions)

111. Also called the birth canal, this is a muscular passageway from the cervix to the outside of the female's body.
- A. Fallopian tube
 - B. Vagina**
 - C. Ovary
 - D. External os
112. A sac shaped like an upside down pear with a thick lining and muscles in the pelvic area where a fertilized egg or zygote comes to grow into a baby
- a. Fallopian tube
 - b. uterus**
 - c. ovary
 - d. vagina
113. The fleshy outer part of the female reproductive system where the opening to the vagina is located.
- A. Vestibule
 - B. Pudenda**
 - C. Labia majora
 - D. Labia minor
114. Which part of the pelvis has no role in labour?
- A. False pelvis.**
 - B. True pelvis
 - C. Pelvic cavity
 - D. A and C
115. What is the most important landmark on the fetal skull?
- A. Temporal fontanel
 - B. Sagittal suture
 - C. anterior fontanel
 - D. Posterior fontanel**
116. Of the following diameter of fetal skull which one is most useful to estimate gestational age of the baby
- A. SMB
 - B. MV
 - C.SOF
 - D.BPD**
117. The smallest interospineous distance of the out let that the baby be able to pass without difficulties is
- A. 8.5cm
 - B. 9.5cm**
 - C.10cm
 - D.10.5cm
118. Uterine Cervix:
- A. Is the lower 1/3 portion of the uterus**
 - B. External OS cell lining is columnar epithelium
 - C. Laterally is attached to the round ligament
 - D. The cervical canal is covered with stratified squamous epithelium
119. When a female pelvis said to adequate for child birth
- A .sacral promontory is reachable
 - B .ischial spine is prominent

- C. supra pubic angle measures 90 degree or above
D. Subpubic arch accept 4 fingers
120. The pelvis includes which of the following bones:
A. Trochanter, hip socket, ischium, sacrum & pubis.
B. Ilium, ischium, pubis, sacrum & coccyx.
C. Ilium, ischium & pubis, fumer.
D. Trochanter, sacrum, coccyx, ilium & pubis.
121. The joint between the two pubic bones is called the:
A. Sacroiliac joint.
B. Pubis symphysis.
C. Sacrococcygeal joint.
D. Piriformis.
122. The greatest diameter of the fetal head is:
A. Occipitofrontal.
B. Occipitobregma.
C. Suboccipitbregmatic.
D. mento vertex
123. Molding of the fetal head:
A. Usually cause brain damage.
B. Becomes progressively easier as gestational age increase.
C. Increase the difficulty of delivery.
D. Does NOT have time to occur in breech delivery.
124. The main blood supply of the vulva is:
A. Inferior hemorrhoidal artery.
B. Pudendal artery.
C. Ilioinguinal artery.
D. Femoral artery.
125. The following are typical in the female bony pelvis EXCEPT:
A. Has a transverse diameter of the inlet greater than the antero-posterior diameter.
B. Has an obstetric conjugate of 11-12 cm.
C. Is funnel shaped?
D. Has a pubic angle greater than 90 degrees.
126. Regarding fetal head, Choose the CORRECT answer:
A. it is composed of face, vult and neck
B. the largest transverse diameter measures 9.5cm
C. Can be delivered vaginally in mento vertex diameter presentation.
127. The ovarian artery is a branch of:
A. Common iliac artery.
B. Internal iliac artery.
C. Abdominal Aorta
D. Hypo gastric artery.
128. Which one of the following is true regarding uterine tubes?
A. It is a place where implantation takes place
B. Interstisium is the widest part
C. approximately it is about 10 cms long
D. supplied by abdominal arteries
129. Which set of structures contains erectile tissue in females?
A. Clitoris and labia majora
B. clitoris and monis pubis
C. vestibular gland and vestibular bulb
D. Clitoris and vestibular bulb
130. The term used to refer to the area encompassing the female external genitalia is?

A .vulva
B .prepuce

C .labia major
D. vestibule

Part V. Reproductive Health and Family Planning (31Questions)

131. Young people tend to be at higher risk of contracting STIs/HIV/AIDS, because of
- A. Intercourse often is unplanned or unwanted.
 - B. Often do not plan ahead for condom or other contraceptive use.
 - C. The immaturity of their reproductive system.
 - D. Little knowledge of STIs
 - E. All
132. Two of MDGs are directly related to the development of midwifery and midwives. What are these two MDGs respectively?
- A. Promote gender equality and empowerment vs. improve maternal health
 - B. Promote gender equality and empowerment vs. reduce child mortality
 - C. Reduce child mortality vs. improve maternal health
 - D. Improve maternal health Vs combat HIV/AIDS
133. The definition of RH according to ICPD includes the following concepts except,
- A. It addresses a state of complete physical mental & social well being
 - B. It is merely the absence of disease or infirmity,
 - C. It refers to all matters related to the reproductive system
 - D. It refers to the reproductive functions & processes
 - E. None
134. Which one is **wrong** about reproductive tract infections?
- A. STIs are Infections transmitted through sexual activity with an infected partner
 - B. Iatrogenic infections get into the reproductive tract by clinical procedure
 - C. Endogenous infections are contracted from unsterilized instruments
 - D. Endogenous infections are those resulted from normal flora found in the vagina
 - E. None
135. Which one is **wrong** about basic emergency obstetric care?
- A. Provided at the health centre level.
 - B. The care should include parenteral antibiotics.
 - C. Manual removal of placenta is done when necessary.
 - D. Includes manual removal of retained products.
 - E. Obstetric Surgery has to be included.
136. Ms Ayelech comes to the nearby health center looking for family planning method, *VCT* and advice on how to feed her young infant. The provider on duty gave her the FP method she want and

told her to return back whenever she feels discomforts. Subsequently she has undergone HIV testing after thorough pretest counseling by the same provider and her result was negative. Mr Provider referred her to < five clinic which is found in the same center for the advice on infant feeding. What levels of service integration do you think in place in this health center?

- A. Facility-level integration
- B. Systems-level integration
- C. Provider-level integration
- D. A & B
- E. A & C

137. Safe mother initiative was the program that initiated in Nairobi in 1987 to half maternal mortality by year 2000. All of the following are components of this initiative; except

- A. Safe and clean delivery
- B. Safe postnatal care
- C. Family planning
- D. Intensive neonatal care

138. Not an indication for removal of IUCD

- A. When the client makes a firm request.
- B. When there is a medical indication for removal
- C. When the effective life span of the IUD expired
- D. When the women reaches menopause
- E. None of the above

139. True about Emergency Contraception?
- A. It prevents possible unwanted pregnancy from unprotected intercourse
 - B. It increase needs for abortion
 - C. ECPs can be used by women in the first five days following unprotected sex
 - D. None of the above
140. Who could use ECP?
- A. All women who are not eligible for pills can use ECPs
 - B. All women who are not eligible for IUDs can use IUCD as EC
 - C. All mothers who have child > 6 months
 - D. All breastfeeding mothers
 - E. Pregnant mothers
141. Surgical Method of contraception
- A. It involves surgical procedures to remove testicles/ fallopian tube
 - B. It is permanent method of contraception done both for men and women
 - C. They prevent transmission of STIs, including HIV
 - D. They do affect sexual function and sexual drive
 - E. B and C
142. Which one is correct about the clinical manifestation of primary syphilis?
- A. Average incubation period is 3weeks
 - B. Chancroid is the typical lesion manifested during primary syphilis
 - C. Chancre at late stage is characterized by maculo-papular lesion
 - D. Chancre at early stage is clean based, painless and indurated ulcer
 - E. Chancroid resolves in 10-90days
143. The criteria for LAM are:
- A. Fully or nearly BF, less than 6 months postpartum, menses not yet returned and baby is fully or nearly fully breastfed and is fed often, day and night
 - B. Fully or nearly BF, less than 6 months postpartum, menses not yet returned and baby still feeds at least once during the day
 - C. Fully or nearly BF, less than 4 months postpartum, menses not yet returned and baby is fully or nearly fully breastfed and is fed often, day and night
 - D. Fully or nearly BF infant who is 6 weeks or less
 - E. None of the above
144. Family planning:
- A. Can prevent pregnancies in women past the desirable child-bearing age
 - B. Facilitates love and affection by parents to the children
 - C. Reduces maternal and child mortality
 - D. Contributes to the quality of family life and economic development
 - E. All except 'A'
 - F. All of the above
145. Which one is *not* considered as maternal death?
- A. W/ro Aberash Abate was died during child birth due to obstructed labor.
 - B. W/ro Almaz Kebede had a prolonged labor when she was giving birth to her last baby and unfortunately she was died after twelve months of her delivery.
 - C. A primigravidae mother was seriously suffering from Kidney disease and died during her second trimester.
 - D. While a nurse student was managing the third stage of labor, a multigravidae woman was died due to heavy PPH.
146. According to WHO Classification, Adolescents are

- A. 15-24 years old
B. 10-25 years old
C. 20-25 years old
D. 10-19 years old
E. None
147. The length of time a woman has used Depo Provera makes no difference in return to fertility.
A. **True** B. False
148. One of the following modes of HIV transmission not correctly matched with transmission rate per exposure?
A. Blood transfusion → >90% per exposure
B. MTCT → 25-40% per exposure
C. **Unprotected sex → 70-80% per exposure**
D. Needle stick & other medical exposure → <0.5% per exposure
E. None of the above
149. Intrauterine contraceptive devices are not contraindicated for a woman:
A. Who is known or suspected to be pregnant
B. Who suffers from unexplained vaginal bleeding
C. **Who is breast feeding**
D. Who presents with current septic abortion
150. Which of the following is **TRUE** about a good counselor?
A. Uses judgmental approach to his/her clients
B. Discourage the client's questions
C. **Understand the effect of non verbal communications**
D. Use too much information.
151. All of the following hormonal methods of contraception can be provided to breast feeding /lactating women safely **except**:
A. Norplant implants
B. Progesterone only pills/mini pill
C. **Combined oral contraceptive pills (COCs)**
D. Implanon
152. All are **TRUE** about Depo- Provera **except** one:
A. It is injectable contraception
B. It is Depot Medroxy Progesterone Acetate (DMPA)
C. The method should be provided to the client every 3 months
D. **The woman can get pregnancy immediately after discontinuing the method**
153. Which one of the following is **correctly** matched about the method and its maximum protection period
A. Implanon – 3 months
B. **Jadelle - 5 years**
C. DMPA – 3 years
D. IUCD(CuT 380A) -12 months
154. Which one is **false** about Gender concepts
A. **It is universal**
B. Social classification of activities for male and female
C. Can be different across the globe
D. Health service inequality can be the reflection of it
E. All

155. The head of “X” company in Arbaminch town said to the girl that, you apply for a job as an executive Consultant. Although you have experience and excellent qualifications, you are not hired because some of the company’s long-time clients are more comfortable dealing with men. This is an example of
- A. Gender bias
 - B. Gender stereotype
 - C. Gender discrimination
 - D. Gender mainstreaming
156. Delay is one of the causes for maternal mortality, so all are contributing factor for the delay of mothers to deciding seeking care **except**
- A. Failure to recognize complication
 - B. Financial problem
 - C. Hospital procedure
 - D. Educational status
 - E. None
157. Which one of the following is **wrong** about biological and cognitive development of adolescents?
- A. Developing and changing is common
 - B. Cognitive dev’t comes before Biological development
 - C. Adolescents don’t have experiences in thinking and behaving
 - D. None
158. The ratio of male birth with female birth is calculated by using the formula

$$\frac{\text{Number of male births}}{\text{Number of female births}} \times 100$$
- Assume the ratio is > 100, this indicates that
- A. More female birth than male
 - B. More male birth than female
 - C. Male and female births are equal
159. Which one of the following can be a challenge to meet MDGs?
- A. Rapid population growth
 - B. Number in need of education, health care and livable wedges
 - C. Large unmet need for family planning
 - D. All
160. World population cannot be affected by:
- A. Birth
 - B. Death
 - C. Migration
 - D. All
161. The most common cause of neonatal death in Ethiopia is
- A. Infection
 - B. Prematurely
 - C. Birth asphyxia
 - D. Tetanus

162. Which one is *wrong* about Female Genital Cutting?

- A. Is a deeply rooted traditional practice
- B. Causes psychological trauma
- C. Can lead to death
- D. All forms of FGC can lead to dyspareunia
- E. None

Part VI. Epidemiology (11 questions)

163. Prevent the disease before it develops by controlling cause and risk factor constitutes

- a. Secondary prevention
- b. Primary prevention
- c. Tertiary prevention
- d. B and C
- e. None

164. A type of descriptive study design in individual in which status of an individual with respect to the presence or absence of both exposure and diseases is assessed at the same point in time is :

- a. Case-control
- b. Case series
- c. Ecological
- d. Cross-sectional
- e. A and B

165. In a recent survey, investigators found that the prevalence of Disease A was higher than the prevalence of Disease B. The incidence and seasonal pattern of both diseases are similar. Explanations consistent with this observation include:

- a. patients recover more quickly from Disease A than from Disease B
- b. patients recover more quickly from Disease B than from Disease A
- c. patients die quickly from Disease A but not from Disease B
- d. patients die quickly from Disease B but not from Disease A
- e. A and C

166. Which of the following is/are not an Etiological factor/s in the causal concepts of a disease?

- a. Excessive- cholesterol
- b. Ionizing radiation
- c. Concurrent infection
- d. Nutritional Deficiency
- e. None of the above

167. One is true among following sentences

- A. Prevalence measures new occurrences

- B. Incidence measures pre-existing cases
 - C. Prevalence is a function of incidence
 - D. Duration of the disease determines its incidence
168. By the year 2005, there were 800 new cases of breast cancer in Alabama while there were 209 new cases of breast cancer among women in Alaska. Which one of the following statements is true about the incidence of breast cancer in Alaska and Alabama based on the above information.
- A. Higher in Alaska than Alabama
 - B. Higher in Alabama
 - C. Data was insufficient to conclude
 - D. Alaska and Alabama had equal incidence rates of breast cancer
169. Which is the use of descriptive epidemiology
- A. Generating hypothesis
 - B. Tests hypothesis
 - C. Deals with determinants of diseases
 - D. Depicts disease patterns
 - E. A and D

Answer Questions 170-171 based on the following information: Disease X occurred in different kebeles in Gondar town (in a population of 100,000 people with male to female ratio of 1:1) affecting both sexes and all age groups. The number of people affected by disease X is summarized as below.

Kebele	PAR	Cases of Disease X	
		Male	Female
1	30,000	12	7
2	20,000	12	3
3	40,000	4	9
4	10,000	0	1

170. Which of the following is true about disease X
- A. It affected both sexes equally
 - B. It is a disease of males than females
 - C. It affected more younger groups in Kebele 3
 - D. It arose in Kebele 3 and then propagated to other kebeles
171. What is the incidence of disease X in Kebele 3 per 100,000 population?
- A. 23.5
 - B. 63.3

D. 32.5

173. An association is most likely to be causal if

174. An epidemiologist is worried about the ever increasing trend of malaria in a certain locality and wants to estimate the proportion of persons infected in the peak malaria transmission period. If he takes a random sample of 150 persons in that locality during the peak transmission period and finds that 60 of them are positive for malaria which of the following is **correct**?

175. In a study of breast feeding and intelligence, 300 children who were very small at birth were given their mother's breast milk or infant formula at the choice of the mother. At the age of 8 years the IQ of these children was measured. The mean IQ in the formula group was 92.8 compared to a mean of 103.0 in the breast milk group. The difference was significant, $p < 0.001$. Which one is true?

176. Of the following which one is **not correct**?
- A. The power of the study is the probability of getting a statistically significant result
 - B. The power of the study is the probability of rejecting a false null hypothesis
 - C. As the power of the study increases, type II error (β) decreases
 - D. As type I error (α) increases, the confidence level decreases.
 - E. Type I error (α) + type II error (β) = 1
177. Of the following which one is **not correct** concerning the p-value and level of significance?
- A. P – Value is the probability of getting the observed result in the sample purely by chance if the null hypothesis were true.
 - B. The level of significance (α) is chosen arbitrarily before the conduct of the study.
 - C. The p-value is the result observed after the study is completed
 - D. Both p-value and level of significance (α) are probabilities.
 - E. All F. None
178. The center of the distribution of the dataset can be assessed by either of the following, except?
- a. **Range**
 - b. Median
 - c. Mean
 - d. Mode
179. One of the following is not advisable, even impossible to compute for categorical variables as a summary measure?
- a. **Range**
 - b. Standard deviation
 - c. Mean
 - d. All of the above
180. One of the following data collection tools is not appropriate for illiterate respondents.
- a. Interview
 - b. Focus group discussion
 - c. **Self administered written questionnaire**
 - d. Observation
181. Which one of the following not true about probability?
- a. The probability of an event can be between 0 and 1 inclusive
 - b. Binomial experiment has only two possible outcome
 - c. The area under the curve of normal distribution is 0.5
 - d. **The probability of the sample space is 1**
 - e. None of the above

182. One of the following is source of primary data except?
- a. Interview
 - b. Self-administered questionnaire
 - c. Hospital record
 - d. Focus group discussion
 - e. None of the above
183. The study was designed to assess the malnutrition status of under five children in Gondar town. If the researcher consulted you about the sampling method should be used for this study population, what will be the most appropriate sampling techniques you need to advice to be used?
- a. simple random sampling
 - b. Systematic random sampling
 - c. Stratified sampling
 - d. Cluster sampling

Part VIII. Health Service Management (7 questions)

184. Which of the following is false?
- a. SWOT analysis is a strategic planning tool
 - b. Planning is prospectively deciding what you achieve in the future and how you reach there.
 - c. Roster is used for duties planned for different staff members, for different times
 - d. Controlling is closely related to planning, organizing & leading
 - e. None
185. The second step in health planning activities is:
- | | | |
|--|--|---|
| <ul style="list-style-type: none"> a. Situational analysis b. Setting objectives & targets c. Selecting priority problems | | <ul style="list-style-type: none"> d. Designing strategies e. Reviewing obstacles |
|--|--|---|
186. Which of the following is SMART objective except?
- a. District x plan to provide 85% of measles vaccine for <1year children at the end of 2004
 - b. District Y plan to vaccinate all <1year children for pentavalent vaccine at the end of 2004.
 - c. HEW in Keble X plan to provide health education for 70% of community at the end of the year
 - d. HEW in Keble Z plan to improve solid waste disposal system
 - e. None
187. Which of the following is true about essential drug selection
- a. Prevalence of malignant diseases in the community
 - b. Toxogenicity of the drug

c. Prevalent health problems of the community

d. inferiority of the drug

e. Drug availability by multiple compound

f. None

188. Which of the following is true about evaluation

a. Evaluation is only important to improve the program/project

b. Evaluation is only to judge the value or worth of the program/project

c. Evaluation is only important to prove the project

d. All

e. None

189. In which management style, “Do what you like”, will be a supervision procedure

a. Laissez faire

b. Autocratic

c. Anarchic

d. Democratic

e. A & C

190. Which of the following is true in Ethiopian health system?

a. In Ethiopia, in the previous years the health service was mainly provided by the government

b. In sector wide approach, funding agencies support a successful vertical health programs in the leadership of the government

c. In a six-tiered system of Ethiopian health care services, health stations were at lower level than health centers by providing services for about 10,000 populations

- d. Traditional medicine was the major means of health care resource in developing countries including Ethiopia.
- e. A & B
- f. A, C & D

Part IX. Health Assessment (10 questions)

191. A 25-year-old teacher presents to the clinic for evaluation of flank pain, dysuria, nausea, and fever. A urine pregnancy test is negative. A urine dipstick is positive for leukocyte esterase. On physical examination, what would be the most likely sign expected?
- A. Psoas sign
 - B. CVA tenderness
 - C. Rovsing's sign
 - D. Murphy's sign
192. What is the preferred order for examination of the abdomen for a man and non-pregnant woman?
- A. Inspection, auscultation, percussion, palpation
 - B. Percussion, auscultation, palpation, inspection
 - C. Auscultation, inspection, palpation, percussion
 - D. Inspection, palpation, auscultation, percussion
193. You are in the emergency room assessing a patient with abdominal pain and fever. You are performing an abdominal examination to assess for peritoneal signs. Which one of the following is NOT a peritoneal sign?
- A. Rebound tenderness
 - B. Involuntary guarding
 - C. Rigidity of the abdomen
 - D. Voluntary guarding
194. A 15-year-old high school student presents to the clinic with a 1-day history of nausea and anorexia. He describes the pain as generalized yesterday, but today it has localized to the right lower quadrant. You palpate the left lower quadrant and the patient experiences pain in the right lower quadrant. What is the name of this sign?
- A. Psoas sign
 - B. Obturator sign
 - C. Rovsing's sign
 - D. Cutaneous hyperesthesia
195. The most important technique when progressing from one auscultatory site on the thorax to another is:
- A. Top-to-bottom comparison
 - B. Side-to-side comparison
 - C. Posterior-to-anterior comparison
 - D. Interspace-by-interspace comparison
196. Which of the following is incorrect?

- A. Light palpation is important identify abdominal tenderness, muscular resistance, and some superficial organs and masses.
 - B. Deep palpation is important to delineate abdominal masses.
 - C. A protuberant abdomen that is tympanic throughout suggests intestinal obstruction.
 - D. Dullness to the medial aspect of the right mid axillary line raises the question of splenomegaly.
197. A 45-year-old hotel manager is sitting in your clinic exam room, waiting for a routine checkup for hypertension. You enter the room and sit down, making eye contact, and move your body lightly toward the patient. You nod your head as the patient begins to bring you up to date on his life since his last appointment. This interviewing technique is an example of:
- A. Echoing
 - B. Adaptive questioning
 - C. Nonverbal communication
 - D. Empathic responses
198. Ato Tsirarge , who is a 31 years old man brought to the health institution you work with. He opens his eyes when you call his name, takes away his leg when you pinch him and speaks inappropriate words. The sum and level of consciousness according to Glasgow coma scale is
- A. 3,deep coma
 - B. 11, alert
 - C. 10, alert
 - D. 9, alert
199. As you palpate the chest, focus on:
- A. Areas of tenderness
 - B. Respiratory expansion
 - C. Fremitus
 - D. All
200. Which of the following is not true?
- A. Bowel sounds may be heard over a hernia, but not over a hydrocele.
 - B. Never use your thumb, for counting pulse.
 - C. In Cranial Nerve X paralysis, the soft palate fails to rise and the uvula deviates to the opposite side.
 - D. None